



## SHOCK

**Effective:** January 1, 2026

**Replaces:** January 1, 2025

### 1. Patient Care Goals

- 1.1. Initiate early fluid resuscitation and vasopressors to maintain/restore adequate perfusion to vital organs
- 1.2. Differentiate between possible underlying causes of shock to promptly initiate additional therapy

### 2. BLS Treatment

- 2.1. Routine Medical Care – Adult (700-S04)
  - 2.1.1. **Oxygen High Flow** – assist with ventilations as appropriate
- 2.2. Place patient in shock position
- 2.3. Control any obvious bleeding as appropriate
  - 2.3.1. Consider the use of a tourniquet if bleeding is not controlled (700-M17)
- 2.4. Maintain patient's body temperature
- 2.5. If Anaphylaxis is suspected see treatment protocol (700-A12)
- 2.6. If Trauma is suspected see treatment protocol (700-A16)
- 2.7. If Sepsis is suspected see treatment protocol (700-A04)

### 3. ALS Treatment

- 3.1. **Vascular Access (IV) or Vascular Access (IO)**, per procedure (700-M13)
  - 3.1.1. A second IV may be established if appropriate
- 3.2. **500 ml Normal Saline** bolus, to maintain systolic blood pressure (SBP) greater than 90 mmHg
  - 3.2.1. May repeat once if lung sounds remain clear
  - 3.2.2. Titrate IV to maintain SBP 90 mmHg
- 3.3. **Push-dose Epinephrine (0.01 mg/ml)** –
  - 3.3.1. Mix **9 ml Normal Saline** with **1 ml Epinephrine 1:10,000** IV/IO formulation in a 10 ml syringe
    - 3.3.1.1. Label syringe "**Epinephrine 0.01 mg/ml**"
    - 3.3.1.2. Administer **1 ml IV/IO** every 1-5 minutes as needed to maintain SBP > 90 mmHg

### 4. Pertinent Assessment Findings

- 4.1. Decreased perfusion manifested by altered mental status, or abnormalities in capillary refill or pulses, decreased urine output (1 mL/kg/hr):
  - 4.1.1. Cardiogenic, hypovolemic, obstructive shock: capillary refill greater than 2 seconds, diminished peripheral pulses, mottled cool extremities
  - 4.1.2. Distributive shock: flash capillary refill, bounding peripheral pulses

### 5. Key Documentation Elements

- 5.1. Medications administered
- 5.2. Vital signs according to Routine Medical Care – Adult (700-S04)
- 5.3. Neurologic status assessment
- 5.4. Amount of fluid given